

THE CHALLENGES OF HEALTH REGULATION IN NIGERIA: THE BIG PICTURE

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Introduction

The Institute of Healthcare Management, Nigeria, acknowledges that health regulation in Nigeria faces significant challenges. The Institute has therefore prioritized a comprehensive discussion of these challenges in today's conference. This topic aligns with the broader conference theme, "*Healthcare Management in Nigeria: Challenges and Prospects*", and specifically supports the sub-theme on strengthening service delivery, regulation, and training.

Healthcare services are packaged commodities provided by stakeholders within registered, inspected, approved, and licensed facilities. These services are subject to periodic evaluation by relevant health authorities. The aim is to make these services readily accessible to patients for treatment, prevention, or maintenance of health.

Health regulatory agencies are empowered to make regulations under the various Decrees, Acts, or Laws of the Federation of Nigeria (LFN) that establish them. They define and advance the training and practice of professionals in their respective fields. These agencies are government parastatals under the supervision of the Federal Ministry of Health (FMOH), each with its own Governing Board or Council.

A key question arises: *What type of healthcare service is being delivered, at which facility, by which professional, and at what level of competence?* Inspection and accreditation are the primary tools used by regulatory agencies to evaluate these questions.

To carry out these functions, agencies rely on Human Resources for Health (HRH), defined by the World Health Organization (WHO) as:

“Those who promote and preserve health, diagnose and treat diseases, and include health management and support workers who do not provide health services directly” (WHO, 2006).

Like all formal organizations, human resources are the heartbeat of health service delivery. This paper examines the relevance of health regulation in Nigeria through the utilization of HRH in healthcare facilities and the challenges faced.

1. Health Regulatory Agencies in Nigeria

Currently, Nigeria has thirteen (13) healthcare regulatory agencies:

1. Community Health Practitioners Registration Board of Nigeria (CHPRBN)
2. Dental Technologists Registration Board of Nigeria (DTRBN)
3. Dental Therapists Registration Board of Nigeria (DTRBN)
4. Health Records Officers Registration Board of Nigeria (HRORBN)
5. Institute of Chartered Chemists of Nigeria (ICCON)
6. Institute of Public Analysts of Nigeria (IPAN)
7. Medical and Dental Council of Nigeria (MDCN)
8. Medical Laboratory Science Council of Nigeria (MLSCN)
9. Medical Rehabilitation Therapists Registration Board of Nigeria (MRTRBN)
10. Nursing and Midwifery Council of Nigeria (NMCN)
11. Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)
12. Pharmacists Council of Nigeria (PCN)
13. Radiographers Registration Board of Nigeria (PRBN)

These agencies are empowered to make regulations under the relevant Decrees, Acts, or Laws of the Federation of Nigeria (LFN) establishing them. They define and advance the training and practice of their respective professions.

All agencies are government parastatals under the FMOH, each with a separate Governing Board or Council. Their core functions include:

- Registration and licensing of practitioners
- Registration and licensing of practice premises
- Oversight of Continuing Professional Development (CPD) programs

These functions are highly professional and ethics-laden, ensuring compliance with standards of practice.

In essence, the main functions of health regulatory agencies are:

1. **Inspection of premises** where registered professionals practice
2. **Accreditation of training institutions** to ensure compliance with approved Benchmark Minimum Academic Standards (BMAS)

For example, the Pharmacists Council of Nigeria (PCN) was established under Act 91 of 1992 (now LFN No. 17 of 2004) to control and regulate pharmacy education and practice comprehensively.

A more detailed example is the Community Health Practitioners Registration Board of Nigeria (CHPRBN), established under Cap. C19:

Duties of the Board:

- a) Determine and improve the standards of knowledge and skills for community health practitioners
- b) Establish and maintain a register of members, publishing lists periodically
- c) Conduct examinations and award certificates or diplomas, prescribing fees for these services
- d) Perform other functions as conferred by the Act

1.1 The Registrar

In all agencies, the **Registrar**, who serves as the Chief Executive Officer (CEO), is responsible for administration in accordance with Public Service Rules. Recruitment follows a transparent process of advertisement, shortlisting, interviews, and selection.

The Registrar must be a registered and licensed professional in the relevant field. For example, under the MDCN Act:

- The Council appoints a fit medical practitioner or dental surgeon as Registrar
- The Registrar prepares and maintains registers of names, addresses, qualifications, and other relevant details of all registered practitioners

The title “Registrar” is appropriate because this officer maintains the official registers of both members and premises of practice.

1.2 Staff of Regulatory Agencies

Regulatory agencies recruit staff as public officers to assist the Registrar in discharging regulatory functions. Recruitment follows Public Service Rules, as seen in the Nursing and Midwifery Council of Nigeria:

- The Council may appoint employees as needed
- It can advance, confirm, promote, transfer, discipline, or terminate staff appointments

Staff rights and privileges are contained in the “Conditions of Service” for each agency. Staff are public officers, paid salaries and benefits approved by the Office of the Accountant-General of the Federation (AGF), and are subject to retirement in accordance with Civil Service rules.

1.3 Governing Boards/Councils

Each regulatory agency has a Governing Board or Council as a policy-making body. The composition varies across agencies. For example, under the Medical Laboratory Science Council of Nigeria (MLSCN) Act, 2003:

Composition of the Governing Board:

- a) A Chairman, a qualified fellow with at least 5 years post-qualification experience, appointed by the President on the recommendation of the Minister from a list submitted by the Association of Medical Laboratory Scientists of Nigeria
- b) A representative of the Federal Ministry of Health and Social Services
- c) Twelve members representing the six geopolitical zones, with rotational representation

- d) Two registered members from Nigerian universities offering Medical Laboratory Science, on rotation
- e) A representative of private Medical Laboratory Science practitioners
- f) A representative of the Association
- g) A representative from a health-focused NGO representing public interest.

Such is the composition of most of the Boards or Councils. The number of the members on such Boards is produced in the following table:

SN	REGULATORY AGENCY	NO OF MEMBERS ON THE BOARD
1	Pharmacists Council of Nigeria (PCN)	63
2	Medical and Dental Council of Nigeria (MDCN)	61
3	Institute of Public Analysts of Nigeria (IPAN)	20
4	Medical Laboratory Scientists Council of Nigeria (MLSCN)	19
5	Nursing and Midwifery Council of Nigeria (NMCN)	18
6	Institute of Chartered Chemists of Nigeria (ICCON)	17
7	Dental Therapists Registration Board of Nigeria (TRBN)	17
8	Medical Rehabilitation Therapist Registration Board of Nigeria (MR TB)	17
9	Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)	15
10	Dental Technologists Registration Board of Nigeria (DR TBN)	13
11	Radiographers Registration Board of Nigeria (RRBN)	12
12	Health Record Officers Registration Board of Nigeria (HRORBN)-	
13	Community Health Practitioners Registration Board of Nigeria (CHPRN)	11

There is, however, a difference between the **Institute of Chartered Chemists of Nigeria (ICCON)** and the **Institute of Public Analysts of Nigeria (IPAN)** regarding the composition of their Boards and the designation of their chairpersons, who are called **Presidents**.

For ICCON, the President and Vice President are **elected by the Council**. The relevant provision in the Act states:

“There shall be for the Institute a President and a Vice President who shall be Fellows of the Institute, to be elected by the Council and hold office each for a term of two years from the date of election.” (*Chapter 112, Institute of Chartered Chemists of Nigeria Act*).

The same election process applies to IPAN. In contrast, the Chairpersons of the other eleven (11) regulatory agencies are **appointed by the President of Nigeria, Commander-in-Chief of the Armed Forces, on the recommendation of the Honourable Minister of Health**. As noted, ICCON and IPAN remain unique in having their leadership **elected rather than appointed**.

A common feature across these regulatory bodies is the provision in their establishing Acts for the constitution of **Investigating Panels and Disciplinary Tribunals**. For instance, the **Medical Rehabilitation Therapists (Registration, etc.) Act** provides for a **Disciplinary Committee**, stating:

“There shall be established a Disciplinary Committee to be known as the Medical Rehabilitation Therapists Practitioners Disciplinary Committee (in this Act referred to as ‘the Disciplinary Committee’), which shall be charged with the duty of considering and determining any case referred to it by the Panel established by the following provisions of this Act.” (*Chapter M9, Medical Rehabilitation Therapists (Registration, Etc.) Act*)

As regulatory agencies, these Institutes, Boards, and Councils were established to **regulate the education and practice of their respective healthcare professions**. Inevitably, there will be deviations among practitioners that must be corrected to **maintain professional standards**. Regulatory mechanisms act both as **punishment for offenders** and as a **deterrent for potential violators**.

Issues relating to **professional misconduct or ethical violations** are typically investigated by the Panels constituted under the various Acts. Following an

investigation, cases are referred to the **Disciplinary Tribunal**, where hearings are conducted openly. The **Attorney-General of the Federation** appoints assessors to assist Panels and Tribunals during their proceedings.

1. The Role of Health Regulatory Agencies in Advancing Healthcare Delivery Services in Nigeria

Emeritus Professor **Umaru Sheu, FAS**, a renowned medic and academic, summarized the roles of professional regulatory bodies as follows:

“The roles of professional regulatory bodies are usually well-defined and largely similar worldwide. They include:

- Setting minimum entry qualifications for students aspiring to join the profession
- Approving curricula for training in various professions
- Monitoring the training of students to ensure compliance with regulations
- Implementing certification and re-certification programs
- Facilitating Continuing Professional Development (CPD) for practitioners
- Ensuring compliance with professional ethics and moral standards in practice”

(CRHP, 2008)

While each healthcare regulatory agency has its specifics, it is the **team approach to healthcare provision** that translates into excellent service delivery. **Prof. Dennis Agbonlahor**, former Chairman of the Medical Laboratory Science Council of Nigeria (MLSCN), emphasized:

“Today's patient care and healthcare practice is multi-professional and multi-disciplinary. It involves teamwork across many healthcare professions. Recognizing that all participants contribute equally valid knowledge and expertise from their professional and personal experiences is essential to working effectively as a team.” (CRHP, 2011)

At the global level, this **teamwork approach** was further emphasized in 1999 when representatives of the **World Medical Association (WMA)**, the **International Pharmaceutical Federation (FIP)**, and the **International Council of**

Nurses (ICN) created the **World Health Professions Alliance (WHPA)**. In 2005, the **World Dental Federation (WDF)** joined the alliance. Together, these four healthcare professions, representing **25 million professionals worldwide**, aim to **facilitate collaboration among key health professionals, governments, policy makers, and the World Health Organization (WHO)**. Working collaboratively, rather than along parallel tracks, enhances patient care and strengthens healthcare systems globally.

I had the privilege of attending the **First World Health Professions Conference on Regulation** in Geneva, Switzerland, from **May 17-18, 2008**, organized by the WHPA. The theme was *"The Role and Future of Health Professions Regulation"*. The conference fully acknowledged the **critical role of regulatory agencies in maintaining healthcare systems** and safeguarding patients. The importance of **government funding for inspections and accreditation** was repeatedly emphasized during the event.

2.1 Accreditation and Inspection of Healthcare Facilities

Institutions that train healthcare professionals must be **accredited by the relevant health regulatory agencies** to ensure that the curriculum, course content, or BMAS (as applicable) complies with the standards approved by the agencies. Accreditation evaluations may cover programs in **training institutes, Schools of Health Technology, Polytechnics, or Universities**.

A **team of experts** from within the profession is typically assembled by the **Governing Council, Board, or Management of the Regulatory Agency**, depending on the program being evaluated. This evaluation may concern either **new or ongoing programs**. Another form of accreditation applies to **Housemanship or Internship centers**, such as those for medical doctors, pharmacists, and radiographers. These programs usually take place within **secondary or tertiary health institutions** or recognized private healthcare facilities.

Supervising professionals must **actively oversee the trainees**. For instance, the **Pharmacists Council of Nigeria (PCN)** requires a **logbook** signed by the **Supervising Pharmacist** (who must have at least five years of post-registration experience) and countersigned by the **Director of Pharmaceutical Services (DPS)**

of the State Ministry of Health. This applies to all internship centers, whether publicly or privately owned.

Inspections of healthcare facilities ensure that services meet **minimum standards of practice**. Inspections vary in purpose:

- **Licensing inspections:** For new healthcare facilities
- **Compliance inspections:** For registered facilities to ensure adherence to standards and public safety

For example, the **MLSCN Inspector** verifies the license status of laboratory scientists in both private and public diagnostic centers, while the **Pharmaceutical Inspector** inspects manufacturing plants, hospital pharmacies, retail pharmacies, and wholesale or importation companies.

The **Nursing and Midwifery Council of Nigeria (NMCN)** faces a significant challenge in inspecting over **200,000 nurses, midwives, and auxiliary nursing staff**, ensuring that quack practitioners are identified and removed. Similarly, PCN addresses the menace of fake drugs through **regular inspections**. Regulatory agencies must maintain **standards of practice** through consistent accreditation and inspections.

To facilitate this, agencies use **protocols and evaluation tools**, including:

- Evaluation forms
- Standard Operating Procedures (SOPs)
- Monographs and books on Minimum Standards

The PCN, for example, has published the “**Four-Part Compendium of Minimum Standards of Practice for the Assurance of Pharmaceutical Care in Nigeria**”, which serves as both a **practitioner’s guide** and an **inspector’s evaluation tool**.

Inspection teams carry out evaluations, with accreditation occurring periodically and inspections continuously. Agencies such as **MDCN, PCN, and MLSCN** are regularly involved in one form of inspection or another.

In PCN, the **Governing Council** has established **Pharmaceutical Inspection Committees (PICs)** at the state level, led by the DPS of the State Ministries of Health and the Federal Capital Territory (FCT). These committees inspect **retail**

pharmacies, wholesale, and importation pharmaceutical firms. Similarly, **Patent and Proprietary Medicine Vendor License (PPMVL) Committees** inspect outlets for patent and proprietary medicines.

These committees may include **professionals not employed by the PCN.** Most are private pharmacists and pharmacy owners who undergo **intensive training by PCN** and are issued **Pharmaceutical Inspector ID cards.** In contrast, inspections of pharmaceutical manufacturing facilities are conducted **exclusively by public officers** from the PCN and State Ministries of Health.

In the case of the **Radiographers Registration Board of Nigeria (RRBN),** inspections of imaging centers in general and teaching hospitals, whether public or private, are **centrally coordinated from the RRBN Registry in Lagos,** due to the **specialized nature of the instruments** and the **limited number of facilities nationwide.**

Inspectors function as the **“police” of healthcare facilities.** At PCN, inspections are **corrective, not punitive.** Emphasizing punishment may discourage stakeholders from presenting their facilities for inspection.

2.2 Continuing Professional Development (CPD) Programmes

The **competence, expertise, and skills** of healthcare professionals (HRH) must be maintained at an **optimal level.** Regulatory agencies ensure practitioners maintain **high professional competence** to:

- Avoid medication errors
- Maintain confidence among peers and patients
- Effectively collaborate with co-healthcare professionals

As healthcare delivery becomes more specialized, from **primary to tertiary levels,** the need for **lifelong learning** grows. The complexity of healthcare, changing environments, lifestyles, re-emergence of diseases, and resistant microorganisms demands that healthcare professionals **continually update their knowledge and skills.**

Continuing Professional Development (CPD) programs provide this framework. At the **National Council on Health (NCH) meeting in Asaba, Delta State (April**

2010), participation in CPDs and acquisition of necessary credit units became mandatory for all healthcare professionals for license issuance.

CPD emphasizes the **“sharing” aspect of learning**, which enhances understanding. The concept was highlighted at the **National Health Summit of 1996**, which emphasized the need for CPDs due to falling standards in tertiary education.

In 1998, PCN launched the **Mandatory Continuing Professional Education (MCPE)** program, with faculties of Pharmacy or Pharmaceutical Sciences serving as provider centers. The program was later renamed the **Mandatory Continuing Professional Development (MCPD)** program in 2004, engaging **university faculty and private practitioners** as resource persons.

Over time, some pharmacists expressed dissatisfaction, noting that lecture notes were repetitive of undergraduate material and lacked practical application. In response, PCN collaborated with **Partners for Transforming Health Systems (PATHS)** to **revamp course content and implement train-the-trainer programs**, making CPD more practical and impactful.

2.3 The Committee of Registrars of Health Professions (CRHP)

August 19, 2004, marked a landmark in Nigerian and West African healthcare regulation. On this date, **Honourable Minister of Health, Professor Eyitayo Lambo**, inaugurated the **CRHP** with the following objectives:

- Raise ethical standards in healthcare delivery
- Set performance targets for efficiency
- Facilitate CPD for health professionals
- Enlighten the public on health matters
- Harmonize regulatory activities for efficiency
- Promote synergy across government healthcare efforts
- Foster unity and mutual respect among health professionals

In his inauguration address, Minister Lambo stated:

“It is heart-warming that Registrars of all regulatory bodies in the health sector are coming together under one umbrella. This is commendable and should be applauded. I urge you to use this Committee as a springboard to rejuvenate the team spirit that is fast fading away.” (*Lambo, 2004*)

Interconnectivity among health regulatory agencies is **essential for coordinated healthcare delivery**, especially as inspectors from different agencies may operate in overlapping facilities. Collaboration ensures **harmony among healthcare professionals** and optimizes service delivery across **primary, secondary, and tertiary healthcare facilities**.

3.0 Challenges of Health Regulation

Key functions of health regulatory agencies include **accreditation, inspections, CPD, and maintenance of practitioner registers**. Provisions for **Investigating Panels and Disciplinary Tribunals** are crucial for **professional ethics and conduct**. Some major challenges in Nigerian health regulation include:

1. **Inadequate Numbers of HRH** – Limits inspection frequency, quality, and integrity. Governments should recruit, train, and fairly compensate inspectors.
2. **Housemanship or Internship Centre Management** – Graduates require adequate placement for practical tutelage.
3. **Poor Logistics Support for Inspectors** – Agencies need vehicles and communication equipment for effective inspections.
4. **Inadequate Training of Inspectors** – Inspectors must receive national and international training to remain technically competent.
5. **Limited or Poorly Packaged CPD Content** – CPD programs should be practical, delivered by professionals with field experience, and tied to license issuance.
6. **Interference by Professional Associations** – Regulatory functions must remain independent from NGOs or voluntary associations.
7. **Frequent Dissolution of Boards/Councils** – Premature board dissolution undermines policy implementation and supervision.

3.1 Inadequate Numbers of Human Resources for Health (HRH)

A **shortage of HRH** directly affects the **frequency, quality, and integrity of inspections**. Just as the government recruits' personnel for the **Police, Army, Air Force, and Navy**, it must also **recruit, employ, train, and develop inspectors** for health regulatory agencies. Inspectors should be **well-compensated** to maintain focus and avoid compromising integrity. They should **never be drawn from the pool of those being inspected**, except in cases requiring intelligence gathering.

3.2 Creation or Upgrading of Housemanship and Internship Centres

Graduates from **medical schools, faculties of pharmacy, and other relevant healthcare programs** require structured tutelage. Adequate **placement for Housemanship or Internship** is essential to ensure practical training. Universities must admit and forward graduates to properly **accredited centers** to complete their required professional training.

3.3 Poor Logistics Support for Inspectors

Many inspectors **lack essential logistics** to carry out inspections effectively. Regulatory agencies must be **adequately funded** to procure **reliable vehicles, communication equipment, and other necessary tools** to facilitate inspections.

3.4 Inadequate Training and Development of Inspectors

Inspectors often **lack adequate knowledge and technical training** to perform their duties effectively. They should receive **comprehensive training, both locally and internationally**, to remain technically competent and ahead of the facilities they inspect.

3.5 Inadequate Course Content in CPD Programmes

Despite the **National Health Summit's directive sixteen (16) years ago**, some agencies have **not implemented CPD programs**, while others offer content that is **too academic and not practical**. Practitioners who have excelled in their fields should be **engaged as resource persons** for CPD programs.

License issuance or renewal should be contingent upon **successful participation in CPD programs**, ensuring that all healthcare professionals remain fit and proper to practice.

3.6 Interference by Professional Associations

Professional associations are **non-governmental organizations (NGOs)** and **voluntary in nature**. However, some associations have attempted to **usurp the regulatory roles of health agencies** through intimidation, blackmail, threats, or coercion. This is **unacceptable**, as regulatory staff are **public officers trained with public funds** to implement their mandates. Regulatory functions must **remain independent** and not be influenced by professional associations.

3.7 Frequent Dissolution of Governing Boards/Councils

Frequent **dissolution of Governing Boards or Councils** before the completion of their terms disrupts **policy-making and effective management supervision**. Boards and Councils should serve their **full terms**, except in cases where the **Federal Ministry of Health** determines that members are non-performing or lack focus.

4.0 Conclusion

The **relevance of health regulatory agencies in Nigeria** cannot be overstated. While recommendations such as the **Orosanye Committee on Parastatals** suggest potential mergers of some parastatals—including health regulatory agencies—the **fundamental need remains effective regulation and control** of healthcare education and practice.

This can only be achieved through **public officers who are well-trained, accountable, and governed through proper public service channels**. Government should **fully fund all health regulatory agencies**, as the **quality of healthcare accessible to Nigerians**—in both registered and unregistered health facilities—**depends on the effectiveness and efficiency of these regulatory activities**.

References

1. **Acts for the Establishment of all Thirteen Health Regulatory Agencies**
2. CRHP (2008): *The Role of Health Regulatory Bodies in Advancing Healthcare Delivery Services in Nigeria* — Proceedings of a One-Day Advocacy Workshop, Abuja, 2008, 91p
3. CRHP (2011): *Harmony in the Healthcare Delivery System: The Role of Regulatory Agencies* — Proceedings of a One-Day Advocacy Workshop, Abuja, 2011, 75p
4. FRN (2007): *National Human Resources for Health Strategic Plan (2008–2012)*, 2007, 83p
5. Lambo, E. Prof. (2004): *Address by Prof. Eyitayo Lambo on the Inauguration of the Committee of Registrars of Health Professions (CRHP), August 19, 2004, SGF Conference Room, Federal Secretariat, Abuja*